

Policy on Laboratory Quality Assurance and Safety Programme

«Hospital Name»

Document control

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Prepared by	Laboratory In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

There is an established laboratory quality assurance and safety programme — so that test results are accurate and precise, and laboratory staff work under documented safety controls aligned with the organisation's safety programme.

1. Purpose

This policy says how «Hospital Name» implements a laboratory quality assurance programme covering point-of-care testing (POCT), pre-analytic, analytic and post-analytic phases, internal quality control (IQC), external quality assurance (EQA) / proficiency testing (PT), calibration, corrective and preventive action (CAPA), clinico-pathological meetings, and a laboratory safety programme with Material Safety Data Sheets (MSDS), training and appropriate safety measures including personal protective equipment (PPE) and immunisation.

The chapter intent is that laboratory services are quality-assured and safe for staff and patients.

This policy does not cover laboratory service scope, specimen handling, turnaround times, critical-result intimation, standardised reporting, or laboratory outsourcing — those are covered in other hospital policies.

2. Scope

This policy applies to the laboratory in-charge, pathologist, microbiologist, biochemist, laboratory technologists, POCT operators outside the central laboratory where laboratory QA covers them, and the Quality Coordinator at «Hospital Name».

It covers the laboratory quality assurance and safety programme: QA programme implementation; internal quality control (IQC); proficiency testing and external quality assessment (PT/EQA) participation; clinico-pathological meetings; the laboratory safety programme; training of all personnel in safe laboratory practices; and appropriate safety measures.

Boundaries:

- Service delivery (menu, specimen process, TAT, critical intimation, reporting, outsourcing) is covered in other hospital policies. Quality failures discovered under this policy feed the recall/amendment process when a released report is wrong.
- Organisation safety / occupational health documents own hospital-wide safety governance; this laboratory safety programme is aligned with them and does not replace them.
- HIC owns hospital infection-control and biomedical-waste streams; laboratory PPE and standard precautions here are the laboratory application of those duties.
- ISO 15189:2022 and NABL 112 are guidance references for the QA programme, not this policy's disclaimer statutes.

3. Policy standards

«Hospital Name» implements a documented laboratory quality assurance programme that covers POCT and the pre-analytic, analytic and post-analytic phases; uses IQC to ensure quality of test results; participates in PT/EQA or documented alternate approaches where formal PT is unavailable; documents CAPA for deviations; conducts clinico-pathological meetings at defined intervals; and implements a laboratory safety programme with MSDS, staff training and appropriate safety measures including PPE, immunisation and standard precautions, aligned with the organisation's safety programme.

«Hospital Name» does not release patient results from a measuring system that has failed IQC without documented override and CAPA, and does not leave laboratory staff without required PPE and safety training for the work they perform.

4. Non-negotiable rules

1. Do not operate a test method without verification or validation as required by the QA programme.
2. Do not ignore an IQC failure: stop patient reporting for the affected parameter until IQC is restored or a documented clinical override with CAPA is recorded.
3. Enrol in scheduled PT/EQA for every analyte where a formal programme exists and this hospital performs the test — never skip enrolment.
4. Do not leave laboratory chemicals or hazards without accessible MSDS for the reagents in use.
5. Do not allow laboratory personnel to work without induction training in safe practices and the relevant MSDS for their job.

6. Do not work without required PPE or without documented immunisation status as required for laboratory staff.
7. Staff who see a laboratory QA or safety rule broken report it the same shift to the Laboratory In-Charge or the Medical Superintendent.

5. What we do

5.1 Laboratory quality assurance programme

The laboratory quality assurance programme is implemented. The organisation has a documented programme to ensure accuracy and precision. The programme includes point-of-care testing (POCT) and covers pre-analytic, analytic and post-analytic phases of the testing cycle, including test standardisation, IQC, EQA or inter-laboratory testing, verification and/or validation of test methods, periodic calibration and maintenance of equipment, and documentation of CAPA whenever deviations are observed.

There is a mechanism to obtain feedback from stakeholders to evaluate laboratory services at least once a year — this is the organisation's adopted minimum cycle; ISO 15189:2022 and NABL 112 require stakeholder-feedback mechanisms without specifying a minimum interval.

5.2 Internal quality control

The programme ensures the quality of test results through internal quality control, including performing IQC to ensure precision and repeatability for all test parameters (quantitative and qualitative) and peer review where relevant. The programme includes comparability of results when more than one measuring system is used.

Patient results for a parameter with failed IQC are not released until IQC is acceptable or a documented override authorised by the Laboratory In-Charge or supervising Pathologist/Biochemist/Microbiologist is recorded with CAPA.

5.3 Proficiency testing and external quality assurance

The laboratory participates in proficiency testing / external quality assurance schemes. Based on the EQA/PT evaluation report, the laboratory implements and documents corrective actions for outliers.

Where formal EQA/PT is not a practical option — for example non-availability of a formal national PT programme for the analyte; only few laboratories performing the test; unstable analyte (blood gases, ammonia, G6PD); or control material of the same matrix not available — the laboratory adopts alternate approaches to validate performance.

5.4 Clinico-pathological meetings

The organisation conducts clinico-pathological meetings at pre-defined intervals for correlating histopathology reports with referring clinicians and uses them as a tool for improving quality. Meetings are scheduled at pre-defined intervals — the organisation's adopted default is at least quarterly unless the Medical Superintendent defines a different interval suited to case volume;

NABH requires such meetings at pre-defined intervals without specifying a minimum frequency.

5.5 Laboratory safety programme

The laboratory safety programme is implemented. A laboratory safety manual is available in the laboratory. It addresses safety of the workforce and of equipment, in consonance with identified risks and hazards. The manual incorporates appropriate MSDS. The programme may follow an Occupational Health and Safety Management System approach. It is aligned with the organisation's safety programme.

5.6 Training in safe practices

Laboratory personnel are appropriately trained in safe practices. All laboratory staff undergo training regarding safe practices in the laboratory and in the relevant MSDS. Training-need identification is commensurate with the job description of the staff.

Training is provided at induction and at least annually thereafter, and when new hazards or equipment are introduced — this is the organisation's adopted minimum frequency; NABH requires training in safe laboratory practices without specifying a minimum interval. Training records are held by the laboratory in-charge.

5.7 Appropriate safety measures

Laboratory personnel are provided with appropriate safety measures. Adequate safety measures are available in the laboratory — PPE, dressing materials, disinfectants, fire extinguishers — addressing safety issues at all levels.

All laboratory personnel always adhere to standard precautions. All laboratory staff are appropriately immunised — hepatitis B at minimum; other immunisations as per the organisation occupational-health schedule. PPE for routine work includes gloves, laboratory coat, eye protection, and face shield where splash risk exists. Spill kits are available and staff are trained in their use.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that laboratory QA and safety programmes are resourced and aligned with organisation safety.
Laboratory In-Charge	Authors and keeps current the QA programme, IQC/EQA matrix, safety manual, MSDS set, training records and CAPA log.
Pathologist / Microbiologist / Biochemist	Supervise technical quality in their disciplines; authorise IQC overrides; contribute to clinico-pathological meetings.
Laboratory technologists and POCT operators	Perform IQC as scheduled; follow safety manual and PPE rules; report deviations the same shift.
Quality Coordinator	Audits this policy; tracks CAPA to closure;

	reviews annual stakeholder feedback on laboratory services.
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7. Quality monitoring (RCA → CAPA)

What is monitored:

- QA programme document current; POCT included; method verification/validation records present.
- IQC performance and CAPA for failures; multi-instrument comparability where applicable.
- PT/EQA enrolment and outlier CAPA; alternate-approach records where PT unavailable.
- Clinico-pathological meeting minutes and follow-up actions.
- Laboratory safety manual and MSDS current; alignment with organisation safety.
- Training and immunisation records complete.
- PPE and safety equipment available and used.

Root-cause analysis is required when the same IQC failure mode or safety incident recurs; the organisation's defined recurrence threshold applies, as documented in the hospital's Root Cause Analysis procedure.

This policy is reviewed annually, and sooner after a serious laboratory incident or major method change.

8. Training and staff acknowledgement

All laboratory staff and POCT operators covered by this programme are trained on this policy at induction and once a year after that. Training covers QA phases, IQC, PT/EQA, CAPA, the safety manual, MSDS relevant to their job, PPE and standard precautions.

Staff acknowledgement

I have read the Policy on Laboratory Quality Assurance and Safety Programme of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

9. Distribution

Official master copy: office of the Medical Superintendent, «Hospital Name», with the Quality Coordinator.

Copies issued to: laboratory; POCT locations covered by this programme; nursing administration for ward-based POCT operators.

The current version is available to all staff at the front-office policy file and, if the hospital keeps an intranet, at staff intranet / policies.

When a new version is issued, take old copies out of use.

10. Abbreviations

Abbreviation	Meaning
AAC	Access, Assessment and Continuity of Care (NABH HCO chapter)
CAPA	corrective and preventive action
EQA	external quality assurance
HCO	Hospital (Full Accreditation programme)
IQC	internal quality control
MSDS	Material Safety Data Sheet
NABL	National Accreditation Board for Testing and Calibration Laboratories
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	objective element
POCT	point-of-care testing
PPE	personal protective equipment
PT	proficiency testing
QA	quality assurance

11. Traceability to NABH HCO Full Accreditation 6th Edition AAC.7

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
AAC.7.a	Commitment*	The laboratory quality assurance programme is implemented.	Section 3; 5.1	Laboratory In-Charge (own programme); Quality Coordinator (annual review)
AAC.7.b	Commitment*	The programme ensures the quality of test results through Internal quality control.	Section 3; 5.2	Laboratory technologists (perform IQC); Laboratory In-Charge (CAPA); supervising Pathologist/Biochemist/Microbi

				ologist (overrides)
AAC.7.c	Commitment	Laboratory participates in proficiency testing / external quality assurance scheme.	Section 3; 5.3	Laboratory In-Charge (enrolment and alternate approaches); Quality Coordinator (audit)
AAC.7.d	Commitment	The programme addresses the clinico-pathological meeting(s).	Section 3; 5.4	Pathologist (convene); referring clinicians (participate); Laboratory In-Charge (minutes)
AAC.7.e	Commitment*	The laboratory safety programme is implemented.	Section 3; 5.5	Laboratory In-Charge (safety manual); Medical Superintendent (alignment with organisation safety)
AAC.7.f	Commitment	Laboratory personnel are appropriately trained in safe practices.	Section 3; 5.6	Laboratory In-Charge (training); staff (attend)
AAC.7.g	Commitment	Laboratory personnel are provided with appropriate safety measures.	Section 3; 5.7	Laboratory In-Charge (provide measures); staff (use PPE and standard precautions)

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

AAC.7.a — QA programme implemented

- Documented QA programme covering POCT and pre-/analytic/post-analytic phases.
- Method verification/validation records; calibration and maintenance schedule.
- Annual stakeholder feedback on laboratory services; CAPA for deviations.

AAC.7.b — Internal quality control

- IQC schedules and results for quantitative and qualitative parameters.

- Comparability records when more than one measuring system is used.
- CAPA and documented overrides for IQC failures.

AAC.7.c — Proficiency testing and EQA

- PT/EQA enrolment matrix by analyte.
- EQA/PT evaluation reports with corrective actions for outliers.
- Documented alternate approaches where formal PT is unavailable.

AAC.7.d — Clinico-pathological meetings

- Meeting schedule at pre-defined intervals.
- Minutes with cases, correlations and discrepancies.
- Follow-up actions linked to quality improvement or CAPA.

AAC.7.e — Laboratory safety programme implemented

- Laboratory safety manual available in the laboratory.
- MSDS set for reagents in use.
- Annual review record and alignment note with organisation safety programme.

AAC.7.f — Training in safe practices

- Training-need identification by job description.
- Induction and annual safe-practice training records.
- MSDS training records for reagents handled by each role.

AAC.7.g — Appropriate safety measures provided

- PPE and safety-equipment inventory (including spill kit and extinguishers).
- Immunisation records for laboratory staff.
- Observation or audit notes on adherence to standard precautions.

13. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Guidebook to Accreditation Standards for Hospitals, 6th Edition — Access, Assessment and Continuity of Care (AAC), standard AAC.7.
- International Organization for Standardization. (2022). <https://committee.iso.org/standard/76677.html> ISO 15189:2022 — Medical laboratories: Requirements for quality and competence. (Official page; full standard text requires purchase.)
- National Accreditation Board for Testing and Calibration Laboratories (NABL). (n.d.). NABL 112 — Specific Criteria for Accreditation of Medical Laboratories. <https://www.nabl-india.org> (official body site; document not separately available as a free public download).

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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